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ON A PARTICULARLY cool June night in Meerut, Uttar Pradesh, 60-year-old Ashish Jha found himself reeling from severe chest pain. A visit to a nearby hospital didn't help. His insurance policy, meant to be a lifeline, was unable to provide cashless treatment for his heart surgery as the hospital lacked the facilities for cashless claims settlement.

94 | Cashless treatment, where the health insurer settles the patient's bills directly with the hospital, has been a pet project of the Insurance Regulatory and Development Authority of India (Irdai). It has raised the bar further, mandating that all cashless claims be processed within three hours of the receipt of discharge authorisation and has asked insurers to put necessary systems in place by July 31. But as the example above illustrates, the initiative faces significant hurdles. Per the latest Irdai report, 42% of the 23.36 million claims in 2023 were settled through reimbursements.

Recently, the Irdai introduced a series of reforms to enhance policyholders' access to and benefits from health insurance. These changes specifically address concerns such as quick cashless treatment everywhere, pre-existing disease coverage, and inclusion of senior citizens.

The question now is, will these reforms lead to a smoother experience for policyholders, or will they be presented with more challenges?

OBSTACLES GALORE

When Jha delved deeper into the issue he faced, he found that it stemmed from the complicated back-and-forth between insurers' claim settlement teams (in-house/third-party administrators), hospitals, and patients.

To help with such eventualities, Irdai has asked insurers to set up dedicated help desks in hospitals and that seems to be a big problem as it requires robust infrastructure and building a network of hospitals is time consuming. Some urban hospitals already have partnerships with insurers and D2C health consulting services (for example, Artivatic.AI, an AI insurance platform, provides rapid claim settlements within 20-30 minutes), but extending this facility to smaller healthcare centres will necessitate significant upgrades.

"I think the three-hour claim settlement is only applicable for tie-up hospitals as all hospitals don't have partnerships with a health insurer," says Naval Goel, Founder and Chief Executive Officer of insurance portal PolicyX.com.

Another change that Irdai has brought in is the reduction in waiting periods for pre-existing diseases (PEDs). But policyholders might still face out-of-pocket costs. "The move has reduced the waiting period by one year, but policyholders have to bear all the expenses themselves for three years for PEDs," says Goel.

WHAT'S THE RUB?

A recent study by insurance marketplace Policybazaar, titled 'Is India happy with health insurance claims?', reveals a troubling trend. Nearly 70% of individuals with reimbursement claims were forced to rely on emergency funds.

While the new regulation allows

BOTTLENECKS IN CASHLESS FACILITY



LACK OF AWARENESS: Many policyholders may not be aware of their cashless entitlement or the process for availing it



INFRASTRUCTURE CHALLENGES: Limited internet connectivity and a lack of insurer tie-ups with hospitals in rural areas can hinder cashless transactions



POTENTIAL FOR FRAUD AND ABUSE: New systems can introduce vulnerabilities, requiring safeguards to prevent misuse by patients, hospitals, or third-party administrators



SECURITY CONCERNS AROUND PATIENT DATA: Cashless transactions involve sharing a patient's medical information, necessitating strong protocols to protect privacy



HIGH COST OF IMPLEMENTATION: Developing and maintaining cashless infrastructure requires investment from insurers, hospitals, and technology providers